



OFFICIAL

Mental Health Restraint Policy

1. Applicability

Site	Operational Area	Applicable to
Armadale Health Service	Mental Health Service	All workforce groups

2. Purpose

The purpose of this policy is to describe safe clinical and documentation requirements should restraint be required to ensure the safety of the patient, staff or others within the Armadale Health Service Mental Health Unit (MHU) in accordance with the [Chief Psychiatrist's Standards for Clinical Care](#).

3. Scope

This policy applies for the authorised MHU at Armadale Health Service and all healthcare workers that work within the MHU.

This policy does not apply to the following:

- Persons who are not patients of the health service (e.g. visitors, relatives and friends)
- Persons under arrest or a prisoner of WA Police or Department of Correctional Services where statutory requirements exist and obligation to public safety and maintaining custody, override medical need
- Discharge against medical advice patients – refer to the [EMHS Discharge Against Medical Advice Policy](#).

4. General principles

The following principles are applicable to the use of restraint in the MHU:

- The use of restraint is not considered a therapeutic intervention and should be avoided wherever possible.
- Armadale Health Service MHU does not support the use of mechanical restraint.
- Restraint must be used in the most respectful way and can only be considered when:
 - There is an imminent risk of danger to the individual or others and no other safer intervention is identified at the time; or
 - There is a need for the administration of a prescribed treatment and all attempts of all other least restrictive alternative interventions have been unsuccessful.
- The use of restraint potentially creates significant risks for people with mental illness, their carers and staff. These risks may include physical harm and/or psychological trauma, loss of dignity, cultural harm, breakdown in therapeutic relationships and injury.

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- Staff must be aware of the persons' risk factors which may result in complications during and following the use of physical restraint (e.g. respiratory problems, cardiac conditions, pregnancy, re-traumatisation, [sexual safety considerations](#) etc.).
- Do not use physical restraint in a way that interferes with the person's ability to communicate, for example by obstructing the eyes, ears or mouth
- Aim to preserve the person's dignity and safety as far as possible during physical restraint
- Ensure that the level of force applied during physical restraint is justifiable, appropriate, reasonable, and proportionate to the situation and applied for the shortest time possible
- The person must be continually assessed to monitor for signs of physical deterioration. The restraint will be ceased immediately if the patient becomes physically compromised
- Prone restraint should be avoided wherever possible. If required to safely manage the person, prone restraint must not exceed three (3) minutes due to an increased risk of hypoxia to the person – consider the use of seated de-escalation or kneeling or standing/walking restraint if safe to do so.
- If staff are concerned about escalation of a potentially dangerous situation, a [Code Black](#) should be called early to ensure prompt attendance by nominated staff and security personnel.
- Mental health consumers, and with consent, their family/carer or nominated Personal Support Person, will be actively involved in the assessment process and their ongoing care planning and treatment choices.

5. Pre-restraint considerations

- On admission, clinical staff will ensure a risk assessment is undertaken to identify any potential for aggression or violence by the mental health consumer.
- A Treatment, Support and Discharge Plan (TSDP) will be completed with the patient and carer/family member as soon as practicable on admission, identifying, wherever possible, precipitating and exacerbating factors which may result in restraint and outline a graded series of responses – complete the Consumer Safety Plan with the patient and incorporate findings into the PSOLIS Management Plan.
- Wherever possible restraint should be planned, and the roles of Lead Clinician (Number 1), Scribe, Number 2, Number 3, and Number 4 identified. In an emergency, when a restraint is unable to be planned, staff are to assume the roles stated above as soon as practically possible during the restraint.
- Before undertaking a restraint, consideration should be given as to whether:
 - The person is medically fit to be restrained
 - It is safe to do so based on assessing the number and appropriateness of staff available (including gender mix, competency, size, physical health etc.)
 - It is suitable to restrain given the persons' risk profile and presentation
 - It is safer to wait for more staff to become available, or to call WA Police or other outside assistance
 - Evacuation is the safest option.
- Medical Practitioners, mental health practitioners or the person in charge of the ward must explore all least restrictive interventions prior to restraint being considered. Prevention and early interventions to consider include, but are not limited to:

- Implementation of the mental health consumer and/or family/carer's documented advice regarding the appropriate strategy to reduce distress and agitation – refer to the documented Consumer Safety Plan
- Defusing and de-escalation techniques
- Peer support involvement, dependent upon consent of the patient and availability of the Peer Support Worker
- Ensuring a culturally secure environment. This may include, if safe and appropriate, consultation with Aboriginal Mental Health Workers to communicate patient options
- Diversional activities and sensory modulation
- Low stimulus environment (e.g. comfort room, outdoor area, dedicated low stimulus zone)
- PRN medication, used in conjunction with other strategies, for the purpose of calming, not sedating.

Restraint exclusion criteria

- Restraint may not be used where the mental health consumer has been assessed as medically unfit
- Restraint may not be used for the purposes of discipline, coercion or staff convenience such as managing inadequate staffing levels.

6. Authorising restraint

All forms related to restraint must be completed as per the [Reporting Restraint to the Chief Psychiatrist Flowchart](#).

If the mental health consumer needs to be restrained urgently:

- A medical practitioner, mental health practitioner or the person in charge of the ward can make an oral authorisation (Form 10A) or written authorisation (Form 10B) which allows the person to be restrained for 30 minutes
- As soon as possible, and within 30 minutes, the person who authorised the restraint (if not a medical practitioner) must inform a medical practitioner that restraint is being or has been used.
- The most senior registered nurse on the ward (after hours the most senior registered nurse on duty) will be informed and is required to attend the restraint as soon as possible.
- The person who authorised the restraint must inform the Treating/Duty Psychiatrist within 30 minutes of the restraint and record the treating psychiatrist's name and qualifications and the date and time they were informed on the restraint form (Form 10C).
- As soon as practicable after the restraint has been applied (or the patient has been released if the restraint is for a short time), the person who authorised the restraint must complete a restraint form (Form 10B).

Clinician alert:

If a restraint is a planned event because a mental health consumer will not accept treatment and needs to be restrained for treatment to be given, a medical practitioner, mental health practitioner or person in charge of a ward will complete a written authorisation for restraint (Form 10B).

7. Restraint to administer prescribed treatment

In the event a restraint is ordered by the Medical Practitioner to enable the administration of a prescribed treatment which is deemed necessary, the Medical Practitioner will:

- Discuss and explain the need for the treatment with the mental health consumer and their carer, nominated Personal Support Person and/or peer support worker
- Determine reasons why the patient may not be accepting the treatment and offer a second opinion where no agreement can be reached
- Ensure all least restrictive alternatives are attempted, right up to the last moment, and documented in the patient's medical record
- Authorise the restraint; and
- Be present during the restraint.

Restraint is only to be authorised when there is documented evidence of the above steps being undertaken.

8. During restraint – patient care management

- The Lead Clinician in collaboration with the restraint team is responsible for ensuring the mental health consumer's wellbeing is continually assessed and especially the mental health consumer's airway and breathing is not compromised.
- Observations of the mental health consumer during restraint must be recorded on the Form 10D and must ensure that:
 - The person has a clear airway
 - The person's limbs are not being injured and
 - The restraint is not causing pain.
- Following the decision to commence a restraint, staff must check and document on the person's observation chart the following at 15 minutes from commencement of the restraint, and then as a minimum hourly thereafter as directed by the medical practitioner:
 - Blood pressure
 - Heart rate
 - Respiratory rate
 - Temperature
 - Level of consciousness
 - Oxygen saturation
 - Pain assessment.

- A mental health practitioner, registered or enrolled nurse must provide ongoing care, utilise verbal de-escalation techniques and ensure mental health consumer's needs and dignity are met.
- A Scribe will be nominated during a planned restraint and is responsible for recording in real time details of the restraint on the Physical Restraint Form (Form 10D) – a Scribe may be self-nominated in an emergency restraint.
- The most senior registered nurse on the ward (after hours the most Senior Registered Nurse on duty) will be informed and is required to attend the restraint as soon as possible.
- In accordance with the mental health consumer's wishes and care plan, the family/carer, or Personal Support Person should be informed as soon as practical regarding the use of restraint and when it has been terminated. This contact is to be documented in the person's healthcare record, identifying the family/carer or Personal Support Person and the time of the contact. If the mental health consumer does not wish to have the responsible person informed this is to be documented in the patient's healthcare records.

9. Amendments to a restraint order

- Variations to the bodily restraint order are to be recorded (Form 10F).
- If the restraint is prolonged, a Medical Practitioner will need to examine the mental health consumer and confirm if the restraint can continue beyond the first 30 minutes (Form 10E). Restraint can only be ordered for periods of 30 minutes at a time and the mental health consumer must be examined by a Medical Practitioner before any extension.
- If the restraint continues for more than six (6) hours, a psychiatrist must review the authorisation for restraint and make a record of that review (Form 10H).
- If the restraint is to be revoked, the Medical Practitioner will complete Form 10G and ensure an examination is carried out (Form 10I).
- Following all restraints, the Medical Practitioner will, within six (6) hours of release, conduct an examination to ensure the mental health consumer has not suffered any complication or deterioration of their mental or physical state (Form 10I).

10. Post restraint management

- Following restraint, all patients must be offered support and counselling preferably from a person of their choice. Support may also include access to the Mental Health Advocacy Service.
- Aboriginal MH staff should ensure the cultural welfare and safety of the mental health consumer following restraint wherever possible
- A Post Restraint Interview must be offered to the mental health consumer and is to be conducted as soon as practicable by a member of staff chosen by the patient. With the consent of the patient, Peer Support involvement and the person's family/carer and/or PSP should be included in the review process.
- The information gathered by the Post Restraint Interview is to be recorded as 'Post Restraint Interview' in the patient's medical record.
- The most senior registered nurse on the ward (after hours the most Senior Registered Nurse on duty) is responsible for ensuring the patient's risk assessment and management plan is reviewed and updated as part of the clinical review process in

collaboration with the mental health consumer and/or their family/carer or Personal Support Person as soon as practicable post restraint.

- Mental health consumers have a right to refuse to engage in a Post Restraint Interview, but every attempt should be made to engage them in the process. If a mental health consumer chooses not to engage in a Post Restraint Interview, this should be documented in the healthcare record.

10.1 Staff debriefing

Line Managers must organise post restraint debriefing for staff in accordance with the EMHS Post Incident Management Policy. Post incident debriefing should include where practicable, lived experience workers.

11. Documentation

- The relevant MHA 2014 Forms must be completed for every occasion of bodily restraint.
- All forms must be checked to ensure they are comprehensively and accurately completed and comply with the MHA 2014 legislative requirements prior to submitting to the Office of the Chief Psychiatrist (OCP).
- A copy of all restraint forms are to be forwarded to the Nurse Manager or delegate once all of the forms are checked and completed in full.
- A copy of all the forms used in the restraint process must be given to the patient unless it is determined by the medical officer that to do so may not be in the best interest of the mental health consumer or others. If the forms are not provided to the mental health consumer, the medical officer will document the reasons for this decision in the mental health consumer's healthcare record.
- Datix CIMS online notification form must be completed whenever the restraint has resulted, or may have resulted, in harm to the mental health consumer.
- The patient's management plan will be reviewed and updated after every incident of restraint.
- PSOLIS alerts and incidents will be reviewed and updated.
- Transfer and discharge documentation should include triggers that resulted in restraint and known individual de-escalation and preventative strategies.

12. Executive review of restraint

An executive review of a seclusion by the HoD Psychiatry, Coordinator of Nursing-Mental Health, and the Nurse Unit Manager must occur within seven days of the event occurring and outcome recorded via the [Seclusion and Restraint Evaluation Tool \(SecRET\)](#).

Where required, a formal case conference can be coordinated for complex cases, which may include:

- Mental health consumer who have three or more restraint events within one week
- Any restraint longer than six hours
- Any event which results in an injury to the mental health consumer, staff or others.

If required, the HoD Psychiatry can further escalate complex cases to the Service Director Mental Health for an independent review.

The HoD Psychiatry will ensure evidence of local interventions and reviews are available during the independent review.

13. Education and training requirements

The following principles are to be included for education and training programs regarding restraint practices:

- Provision of staff access to, and attendance at, initial prevention and de-escalation training within six weeks of commencing work within the MHU, and annually thereafter in accordance with the [EMHS Mandatory Training Policy](#).
- Whenever possible, training is inclusive of the person with a lived experience of mental illness and/or family and/or personal support persons.
- A clear guide to ensure that staff know when an intervention is safe to implement and when to call for assistance. This may include a risk assessment for the potential for aggression and violence, but should focus on the appropriate verbal intervention for people experiencing agitation and anxiety that has not reached the point of acting out
- Understanding the theory of, and antecedents to, aggressive or challenging behaviours to improve prevention strategies and the use of least restrictive practices
- Include language and cultural considerations for Aboriginal people and people from CaLD communities.

Training in the application of physical restraint should:

- Have a strong ethos of avoiding restraint even when a team response is requested
- Be focused on keeping the restraint to the least restrictive possible, for the minimum amount of time required to re-establish safety
- Avoid prone restraint wherever possible and limit it to three (3) minutes when it does occur
- Include consideration of any prior preference expressed by the individual concerned
- Include developing practice and scenario training for teams.

14. Roles and responsibilities

The following outlines the specific roles and responsibilities relevant to this policy:

Role	Responsibility
Person in charge of the ward	• Gradually decrease the number of staff involved • Regular communication with the Lead Clinician and Scribe prior, during (if appropriate), and after the restraint event • Coordinates the safety of the clinical environment during the intervention • Delegation of specific staff roles such as Lead Clinician, Scribe, Number 2, Number 3, and Number 4 • Manages crowd control and medication requirements • Informs the treating Medical Officer • Ensuring the recording of times are consistent across all documentation.

Role	Responsibility
Lead Clinician	• Communicate and support the patient • Direct the restraint team to ensure safe physical restraint techniques are applied • Monitor the patient's physical wellbeing in collaboration with the restraint team • Reduce the intensity of the restraint whenever possible • Gradually decrease the number of staff involved in restraint when safe to do so • Consider the use of alternatives to restraint and/or terminate the restraint if they assess it is safe to do so • Coordinates the restraint event during the intervention • Manages medication requirements.
Scribe	• Documenting the events as they occur in real time on the Physical Restraint Form (Form 10D) • Ensuring the recording of times are consistent across all documentation • Alerting the Lead Clinician of each minute that has elapsed and the length of time that has elapsed since the commencement of any prone restraint until the cessation of the prone restraint.
Security Officers	• Assist the clinical team under the direction of the Lead Clinician, Mental Health Practitioner or Medical Practitioner, or person in charge of the ward in accordance with this policy.
Medical staff	Medical staff must take a proactive role in minimising restrictive practices: • The treating or on-call Psychiatrist must take an active leadership role in facilitating strategies which reduce restrictive practices • Comply with the requirements of the MHA 2014 as they pertain to restraint • Take an active decision-making role early in the restraint process to limit duration and maximise safety.
All staff	• Complete continuing education in least restrictive practice and the use of restraint intervention • Ensure reduction in the use of restraint and seclusion and engage in monitoring of quality care and patient and population health outcomes to inform quality improvement • Ensure documentation and reporting requirements of the MHA 2014 and associated regulations are met • Engage with support, debriefing and opportunities to learn about restraint minimisation

Role	Responsibility
	Aboriginal Mental Health Workers are to be involved following episodes of restraint when it is appropriate to follow up the cultural welfare and safety of the patient.

15. Compliance monitoring

Compliance against this policy will be monitored by the Comprehensive Care Steering Committee.

Compliance monitoring activities relevant to this policy include:

- Episodes of seclusions and restraint (per 1000 bed days as denominator)
- Behaviour-related clinical incident trends (per 1000 bed days as denominator)
- API 3 training compliance rates
- Summary of findings and applicable improvement actions arising from Executive Review of Restraints.

16. Legislative context

The following documents are required to give effect to this policy:

- Legislation
 - *Mental Health Act 2014 (WA)*
 - *Carers Recognition Act 2004 (WA)*
- Chief Psychiatrist of WA
 - [Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist Policy for Public Mental Health Service](#)
 - [sexual safety considerations](#)
- HDWA
 - [Consent to Treatment Policy](#)
- EMHS
 - [Emergency Psychiatric Treatment Policy EMHS: 18](#)
 - [Rights of Patients in Mental Health Policy EMHS: 170](#)
 - [Rights of Carers and other Support Persons Policy EMHS: 163](#)
 - [Mandatory Training Policy EMHS: 51](#)
 - [Notification requirements under the WA Mental Health Act 2014 Policy EMHS: 165](#)
- AKG
 - [Mental Health Seclusion Policy](#)

17. Supporting Documents

- [Code Black](#)
- [Reporting Restraint to the Chief Psychiatrist Flowchart.](#)
- [Seclusion and Restraint Evaluation Tool \(SecRET\).](#)

18. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.33
- Standard 5 – Action 5.34
- Standard 5 – Action 5.35
- Standard 5 – Action 5.36
- Standard 6 – Action 6.09
- Standard 6 – Action 6.10
- Standard 6 – Action 6.11

National Standards for Mental Health Services (2010):

- Standard 1 – Criteria 1.1
- Standard 2 – Criteria 2.1
- Standard 2 – Criteria 2.2
- Standard 2 – Criteria 2.6
- Standard 10 – Criteria 10.5

19. References

1. Office of the Chief Psychiatrist WA 2025. Chief Psychiatrist's Standards for Clinical Care. [online] Available at: <https://www.chiefpsychiatrist.wa.gov.au/standards-guidelines/chief-psychiatrists-standards-for-clinical-care>
2. Office of the Chief Psychiatrist WA 2020. Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia. [online] Available at: <https://www.chiefpsychiatrist.wa.gov.au/publication/chief-psychiatrists-guidelines-for-the-sexual-safety-of-consumers-of-mental-health-services-in-western-australia/>
3. Melbourne Social Equity Institute (2014) Seclusion and Restraint Project: Report, Melbourne: University of Melbourne

20. Glossary

The following definitions are relevant to this policy.

Term	Definition
Bodily restraint	The physical or mechanical restraint of a person who is being provided with treatment or care at an authorised hospital.
Physical restraint	The application of bodily force to the person's body to restrict the person's movement.

Term	Definition
Mechanical restraint	Mechanical restraint: is the application of a device, to restrict the person's movement, such as a belt, harness, manacle, sheet or strap. Mechanical restraint does not include either the appropriate use of medical or surgical appliances or the appropriate use of furniture to restrict a person such as cot sides or a chair fitted with a table. It also does not include physical or mechanical restraint by a police officer.

21. Policy owner

Service Director Mental Health

Enquiries relating to this policy may be directed to:

Title: Coordinator of Nursing - Mental Health
 Department: Mental Health Service
 Email: Dene.Olive@health.wa.gov.au

22. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2.1	23 01 2026	23 12 2028	Minor Review, multiple links updated

23. Authorisation

This policy has been authorised and issued by:

Authorised by	Dene Olive – A/ Service Director Mental Health
Authorisation date	16 01 2026
Published date	23 01 2026